

regulate their fee structure. For instance, despite NEET being introduced as a common admission examination for all medical colleges to check the "selling of seats" in private institutions, the menace continues. In October 2019, an income-tax raid on two deemed-to-be universities in Karnataka blew the lid off precisely such a "seat blocking" scam in private medical colleges. An INDIA TODAY investigation in Uttar Pradesh the same year also exposed how private medical colleges were brazenly ignoring NEET scores by pre-blocking seats for low-ranking MBBS aspirants in exchange for heavy donations.

These corrupt practices certainly warrant more radical reforms than the ones the Union government has currently embarked on, though many invest their faith in market forces weeding out the institutes of poor quality. "The colleges of inferior quality will eventually be eliminated by a process of natural selection," says Dr Asokan. "It happened after engineering colleges mushroomed in the country."

Others advocate stricter regulation of private sector colleges and greater focus on the expansion of medical colleges in the government sector. It is, therefore, crucial that India moves beyond the abysmally low government spending on public healthcare. Despite the festering crisis in its healthcare system, India spends just 1.1 per cent of its GDP on healthcare, which has seen only a marginal increase during the pandemic. "Medical education should be heavily subsidised and preferably be in the public sector," says K. Sujatha Rao, former secretary in the Union ministry of health and family welfare. "The focus has to return to making merit, not the ability to pay, the criterion for admission."

WHAT HAPPENS TO STUDENTS WHO HAVE RETURNED?

On March 4, the IMA wrote to Prime Minister Narendra Modi to ensure that students returning from Ukraine are accommodated in medical colleges in India by increasing 2-5 per cent seats in government and private colleges.

The demand for permitting these students to migrate to Indian colleges to complete their education has also grown after certain other countries decided to make special provisions for these students. For instance, the Gulf Medical University in the UAE has announced that it will accommodate Indian students returning from Ukraine. In a video in which Union minister General V.K. Singh is speaking to students, he is seen announcing that Poland had agreed to admit students rescued from Ukraine in its universities. Many undergraduates are now looking at Egypt, Poland and Armenia as potential destinations to finish their courses.

For now, the NMC has allowed the Ukraine-returned students who got their MBBS degrees but could not complete their internship to complete their remaining internship in India. The Union government has told the Supreme Court that it is considering the issue of further studies for the Ukraine-returned medical students. Under Indian rules, their credits cannot automatically be transferred to Indian medical colleges. A similar situation had arisen after Indian students abroad returned home following Covid lockdowns in countries. The NMC did not offer them any relaxation.

Several experts, in fact, advocate against the seamless transfer to Indian medical colleges as medical education in these countries has often been of questionable quality. Only 16 per cent of overseas medical graduates on an average can clear the FMGE examination. Graduates from Ukraine do slightly better, with a success rate of 17.22 per cent. A student is allowed three attempts; if they still can't make it, they can work as assistants to registered doctors or choose an alternative profession.

Though a cruel analogy to use for the Ukraine-returned students, their concerns need to be addressed on a war footing. The longer-term imperative, though, should be to bridge the demand-supply gap in medical education. The pandemic and the war in Ukraine have only underscored an existing deficit. It is not sufficient to increase the number of seats and medical colleges; imparting quality, affordable medical education should be the top priority instead. And to have both quantity and quality, experts offer a unanimous prescription: planned public investment in healthcare infrastructure and smart and innovative reforms. On this will rest the health of the nation. ■